

## ALOVELL TABLET 70 mg

### Alendronate 70 mg

#### Product Description

White, round uncoated tablet, shallow convex, blank on side I and II.

#### Composition

Each tablet contains Alendronate monosodium trihydrate equivalent to 70 mg of Alendronate.

#### Pharmacodynamics

Alendronate sodium, a synthetic bisphosphonate analog of pyrophosphate is an inhibitor of osteoclast-mediated bone resorption. Alendronate is resistant to enzymatic hydrolysis by phosphatases.

Alendronate appears to inhibit bone resorption in a dose-dependent manner and is 100-1000 times more potent than etidronate in this effect. In addition, alendronate is a highly selective inhibitor of bone resorption; data from animals indicate that the dose of alendronate that inhibits bone mineralization is up to 6000 times greater than the dose that inhibits resorption. Alendronate is incorporated into bone and has an estimated terminal elimination half-life of at least 10 years in humans; however, while incorporated into bone matrix, the drug is not pharmacologically active.

#### Pharmacokinetics

Like other bisphosphonates, alendronate is poorly absorbed following oral administration. Absorption is decreased by food, especially by products containing calcium or other polyvalent cations. Bioavailability is about 0.4% when administered half an hour before food, reduced from 0.7% in the fasting state; absorption is negligible when taken up to 2 hours after a meal. In vitro and in vivo studies have shown that bisphosphonates are absorbed from the gastrointestinal tract via paracellular transport. About half of the absorbed portion is excreted in the urine; the remainder is sequestered to bone for a prolonged period. Bisphosphonates do not appear to be metabolized.

Systemically available bisphosphonates disappear very rapidly from plasma, and are partly taken up by the bone and partly excreted by the kidney. Concentrations of the drug in plasma following therapeutic oral doses are too low for analytical detection (less than 5 ng/mL). Protein binding in human plasma is approximately 78%.

#### Indications

Alovell is indicated for the treatment of osteoporosis in postmenopausal women.

For the treatment of osteoporosis, Alovell increases bone mass and prevents fractures including those of the hip and spine (vertebral compression fractures). Osteoporosis may be confirmed by the finding of low bone mass or by the presence or history of osteoporotic fracture.

Alovell is indicated for the treatment of osteoporosis in men to prevent fractures.

#### Recommended Dosage

Alovell must be taken at least ½ hour before the first food, beverage or medication of the day with plain water only. Other beverages (including mineral water), food and some medications are likely to reduce the absorption of Alovell.

To facilitate delivery to the stomach and thus reduce the potential for esophageal irritation, Alovell should only be swallowed upon arising for the day with a full glass of water and patients should not lie down for at least 30 min and until after their first food of the day. Alovell should not be taken at bedtime or before arising for the day. Failure to follow these instructions may increase the risk of esophageal adverse experiences.

Patients should not chew the tablet or allow the tablet to dissolve in their mouths because of a potential for oropharyngeal ulceration.

Patients should receive supplemental calcium and vitamin D, if dietary intake is inadequate.

No dosage adjustment is necessary for the elderly or for patients with mild to moderate renal insufficiency (creatinine clearance 35-60 mL/min). Alovell is not recommended for patients with more severe renal insufficiency (creatinine clearance <35 mL/min) due to lack of experience.

#### *Treatment of Osteoporosis in Postmenopausal Women and in Men:*

The recommended dosage is one 70-mg tablet once weekly

#### Mode of Administration

Oral

#### Contraindications

Alovell must not be used in patients with :

- Abnormalities of the esophagus or other factors that may delay esophageal emptying.
- Inability to stand or sit upright for at least 30 minutes.
- Hypersensitivity to the drug or intolerance to the components of the formulation.
- Hypocalcaemia.

#### Warnings and Precautions

Alendronate should not be given to patients with abnormalities of the esophagus or other factors that may delay esophageal emptying, or those unable to stand or sit upright for at least 30 minutes. Alendronate also should be used with caution in patients with a history of upper GI disease (e.g., dysphagia, esophageal diseases, gastritis, duodenitis, ulcers). To minimize the risk of esophageal reactions, patients should be in plenty of water (not less than 200 mL), in an upright position (standing or sitting). Tablets should be taken on rising for the day, on an empty stomach, at least 30 minutes before breakfast and should not lie down before eating the first

meal of the day. Alendronate should not be taken at bedtime, or before getting up for the day. Hypocalcaemia should be corrected before starting alendronate therapy.

#### Interactions with other Medicaments

Like other bisphosphonates, Alendronate are not well absorbed from the gastro-intestinal tract, and administration with food further impairs their absorption.

Compounds containing aluminium, calcium, iron, or magnesium, including antacids and mineral supplements and some osmotic laxatives, can also impair the absorption of bisphosphonates given by mouth.

It has been suggested that administration of bisphosphonates with NSAIDs may result an increased incidence of gastro-intestinal and renal adverse effects. There may be additive hypocalcaemic effects with aminoglycosides.

#### Statement on Usage During Pregnancy and Lactation

##### *Use during pregnancy*

Alendronate should not be used during pregnancy. There are no adequate data from the use of alendronate in pregnant women. Animal studies do not indicate direct harmful effects with respect to pregnancy, embryonal/foetal development, or postnatal development. Alendronate given during pregnancy in rats caused dystocia related to hypocalcaemia.

##### *Use during lactation*

It is not known whether alendronate is excreted into human breast milk. Alendronate should not be used by breast-feeding women.

#### Adverse Effects/Undesirable Effects

Abdominal pain has been reported to be the most frequent adverse effect, but is usually mild and self limiting. Peptic ulceration has been reported. Severe esophageal reactions such as esophagitis, erosions, and seek medical attention if they develop symptoms such as dysphagia, new or worsening heartburn, pain on swallowing.

#### Overdose and Treatment

Overdosage with bisphosphonates would be likely to result in symptoms of hypocalcaemia; if necessary, parenteral infusion of a calcium salt could be given. Administration of milk or antacids, to bind the bisphosphonates and minimize absorption, has been suggested for oral overdosage.

#### Storage Conditions

Store below 30°C. Protect from light and humidity.

#### Dosage Forms and Packaging Available

Box, 1 strip @ 4 tablets.

#### Shelf-life

24 months

#### Registration Number

ALOVELL TABLETS 70 mg – MAL16045031AZ

#### Name and Address of Manufacturer

PT. Novell Pharmaceutical Laboratories  
Jl. Wanaherang No. 35, Tlajung Udik,  
Gunung Putri, Bogor 16962 – Indonesia

#### Product Registration Holder

Celestema Pharma Sdn Bhd  
No. 3, Jalan 19/1, Seksyen 19,  
46300 Petaling Jaya, Selangor, Malaysia

#### Date of Revision of Package Insert

14th February 2023